Medical Conversation Analysis Report

Generated on: 2026-09-07 23:30:11

Visit ID: visit-9b8a1ef6

Primary guideline: NICE CG150

Conversation History

patient: Good morning, Marcus. How have you been feeling since our last visit? Mostly okay, but I have been getting these headaches in the afternoon. Usually around 2 or 3pm. How often are these headaches occurring? Daily, a few times a week? Maybe four or five times a week. They last about an hour. On a scale of 1 to 10, how would you rate the pain? Maybe a 5 or 6. Uncomfortable but I can still work.

Analysis and Insights

Key Points

- Recurrent afternoon headaches occurring 4-5 times/week
- Headaches typically last ~1 hour with moderate pain (5-6/10)
- No current medications or relevant medical history reported
- Headaches occur predictably around 2-3pm without trigger identification

Action Items

- Assess caffeine intake history
- Evaluate daily hydration habits
- Screen for screen-time/posture contributing factors
- Consider headache diary for pattern tracking

Patient Concerns

- Frequency of headaches (4-5 times weekly)
- Impact on daily work activities

Medical Terms

- Tension-type headache (bilateral pressure-like headache)
- Caffeine-withdrawal headache (headache from reduced caffeine intake)
- Cluster headache (severe unilateral headache with autonomic symptoms)

Follow Up Questions

- Do you consume caffeinated beverages? If so, timing/quantity?
- Any light/sound sensitivity during headaches?
- Average daily water intake?
- Neck stiffness or visual changes with headaches?

Symptoms

- Afternoon headaches

- Moderate pain (5-6/10)

Medications

• None documented.

Diagnosis Considerations

- Timing suggests possible caffeine/circadian relationship
- Moderate pain makes primary headache disorders more likely than secondary causes
- Lack of red flag symptoms lowers urgency for imaging

Risk Factors

- Potential caffeine dependence (unconfirmed)
- Possible dehydration (unconfirmed)
- Occupational postural strain (unassessed)

Differential Diagnosis

• Tension-Type Headache (69%)

Rationale: Patient presents with recurrent afternoon headaches of moderate intensity, consistent with tension-type headache criteria.

• Caffeine-Withdrawal Headache (65%)

Rationale: Afternoon headaches could align with caffeine withdrawal, given timing and recurrence.

• Dehydration-Related Headache (62%)

Rationale: Afternoon headaches may align with dehydration, though fluid intake is unassessed.

• Migraine without Aura (61%)

Rationale: Moderate intensity afternoon headaches could suggest migraine, though lacking key features like photophobia.

• Cluster Headache (56%)

Rationale: Timing and recurrence could hint at cluster headaches, though severe pain is atypical.